



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : دلال عقبى امين عبدالفتاح
المحافظة : الجيزة
الحى / المدينة : ابوالنمرس
رقم البطاقة : 28008162103489 رقم التليفون : 01069478604
العنوان : منيل شيهه
العمر (تاريخ الميلاد) : 16-08-1980 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: دلال عقيى امين عبدالفتاح
National ID: 28008162103489



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



Patient Name : دلال عقيبى امين عبدالفتاح
 Patient Info. : Female 40Y
 Department : Out Patient
 Profile NO. : 4802-2021

Patient Id : 14591
 VisitId : 144911
 Collection Date : 31/01/2026 09:32:34
 Reporting Date : 31/01/2026 10:27:31

Clinical Chemistry Report

<u>Test</u>	<u>Result</u>	<u>Unit</u>	<u>Reference Range</u>
<u>Kidney Function</u>			
Blood Urea	: 35.3	mg/dL	15.6 - 45
Serum Creatinine	: 0.63	mg/dL	0.5 - 0.9
Calcium Ionized (Ca++)	: 1.22	mmol/L	N. 1.1 - 1.4

Signature

Dr. Sahar Mansour

Name: دلال عقبي امين عبد الفناح		
MR. : 6418		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was diagnosed in April 2022 with a malignant breast tumor staged T1 N1. In May 2022 a multidisciplinary team recommended anti-HER2 therapy together with hormonal agents including PORTH, FEMARA, and ZOLADEX. As of 3 January 2026 the regimen continued with ZOLADEX and FEMARA. On 6 March 2026 the patient was prescribed FEMARA, calcium supplementation (1 tablet daily, 90 days supply), ONE ALPHA, and ZOLADEX 10 mg subcutaneously. The pharmacy was instructed to dispense ZOMETA 4 mg subcutaneously, with a note indicating a free CT chest scheduled for June 2025. On 4 January 2026 the medication list was reiterated: FEMARA, calcium (1 tablet daily, 90 days), ONE ALPHA, and ZOLADEX 10 mg subcutaneously. The plan included a free CT chest in June 2025 with full laboratory workup. On 7 January 2026 the most recent oncology note documented that the last CT chest-abdomen-pelvis performed on 4 January 2026 was free of disease. A repeat scan was scheduled for 6 January 2026, also free of disease. The next ZOMETA dose was planned for September 2026, and the patient continued on FEMARA and ZOLADEX with the next follow-up visit scheduled in three months.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	الطبيب المعالج : ايه مدحت بدوي محمد	
يعتمد ,		



മ. ആരോഗ്യവകുപ്പ്
ആരോഗ്യവകുപ്പ്, മ. ആരോഗ്യവകുപ്പ്

MDT Form

MDT FORM

23/1/2022

Name	23/1/2022	MRIs	C/SI / L.A.S
Age	45 years	ID	C.A. - 11/11/11
Menopausal status	Pre-menopausal	Phone	9845678901
OCP	loop	marital status	married
Comorbidity	-ve	Family history	-ve

Clinical Data			
ER:	PR:	HER2:	Ki67:
8/8	7/8	+3	15%
Immunohistochemistry:			
amplified by ISH			
Metastasis:			
T N M			
Recommendations:			
initially CT/M (multifocal) Recurrence NACT TCHP → PRM → YPIKYPN			

MDT OUTCOME & PLAN

- 1) For TDM x 12 cycles.
- 2) PORT
- 3) Adjuvant Endocrine therapy
Zoladex + AI

Medical Oncologist	Signature	Signature	Signature
Dr. V. S. S. S.	Dr. V. S. S. S.	Dr. V. S. S. S.	Dr. V. S. S. S.

4802
2021
01069478694

Patient Name	: الأ.م.م.ش. / د. ل. ح. / د. ل. ح. / د. ل. ح.	Request No.	: 13009491
Patient No.	: 28008162103489	Request Date	: 24/04/2022
Path No.	: 1111/2022	Verification Date	: 11/05/2022
Sex	: Female	Financial Category	: أورام دار السلام - م.م.م.م.
Age	: 41 Y	Department	: Out Patient
Referring Doc		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data:

Left breast cancer post NACT

Nature of the specimen:

Left MRM specimen, collected at 23/4/2022, 11:10 pm

Gross Examination:

- Procedure: Left MRM specimen measured 18 x 15 x 3 cm, skin covering ellipse measured 18 x 6 cm, detached axillary fat measured 6.5 x 4 cm.
- Tumor extent: A greyish white firm mass
- Size: 1.5 x 1.5 x 1 cm.
- Skin involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 0.5 cm away from deep margin.
- Lymph nodes number and size: 13 LNs up to 1.5 x 1 cm.

Microscopic examination:

- Histological type: Invasive duct carcinoma with micropapillary features
- Grade (differentiation + nuclear pleomorphism + mitosis): GII(3+2+1)
- Tumor extent: (Tumour cellularity up to 35%)
- Angiolymphatic invasion: Present
- Ductal carcinoma in situ: Focal (<5%)
- Skin involvement: Free nipple and areola.
- Lymph node involvement: Positive metastatic deposits in four LNs (4/13). Diameter of largest mets = 0.4cm and one node showing therapy induced changes.
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Therapy changes (Hyalinosis and Fibrosis)

Verified by:

dr. manna atef

Printing Date: 16/05/2022 10:10:03

Page 1 of 2

Printing By: marwa mohamed youss



National Women's Health Initiative Pathology Lab
مركز صحة المرأة - مختبر الأمراض النسائية

Patient Name : الأستاذة / دلال عيسى ابراهيم عبدالقادر
Patient No. : 280008162103489
Path No. : 2578/21
Sex : Female
Age : 41 Y
Referring Doctor :

Request No. : 133365
Request Date : 30/10/2021
Verification Date : 10/11/2021
Financial Category : اورام دار السلام - مبداء
Department : Out Patient
IHC No. :

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast mass

Nature of the specimen:
Tru-Cut biopsy

Gross Examination:
5 greyish white tissue cores 1.5cm each. Totally submitted.

- Microscopic examination:
- Histological type: Partially autolysed specimen, Invasive duct carcinoma
 - Grade II
 - Angiolysis: No
 - Ductal carcinoma in situ: No
 - Lymph node involvement: N/A
 - Resection Margins: N/A
 - Additional pathologic findings: N/A

Diagnosis
Left breast mass; Tru-Cut biopsy:
Invasive duct carcinoma grade II

IHC:
ER: Strong nuclear reaction in more than 70% of tumour cells, Positive, score (8/8)
PR: Strong nuclear reaction in about 60% of tumour cells, Positive, score (7/8)
Her-2: Strong incomplete membrane reaction in 10% of tumour cells. Equivocal, score 2+
Ki67: 15%
Pending for SISH

Prof. Dr. Gamal Abd El Aty
Dr. Abdulrahman Ibrahim

Printing Date : 14/11/2021 10:24:58

Page 1 of 2

Printed By : anwar mohamed youss



رقم المرسوم: 20 ج 15

تاريخ المرسوم: 2024-4-15

جهة الإصدار: فرع الجديدة

المرسوم رقم 20 ج 15

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للتأمين الاجتماعي للتأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "المرسوم رقم 20 ج 15" والارامل " وكذلك قانون 99 لسنة 1992 الخاص بطلب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المتفرغين " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المتفرغة . وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية وطبقا لقاعدة البيانات المرسلة من التأمين الاجتماعي والتحديث لمرور أن

المرسوم رقم 28008162103489 طبقا رقم

وطبقا إلى خطاب من التأمينات الاجتماعية بطلب عدم الاقتناع
بمستوى من الخدمات غير المرضية بالتأمين الصحي هذا الخطاب دون أية مسؤولية على الهيئة العامة للتأمين الصحي حال خروج المواطن من قانون التأمين الصحي الشامل
وأخيرا

تاريخ المرسوم: 2024-4-15



مستند رقم 13/1932

هذا المرسوم صادر في 15 من شهر أبريل 2024 من تاريخ إصداره
والمرسوم رقم 20 ج 15 من تاريخ إصداره
وزارة التأمينات الاجتماعية أو وزارة الشؤون الاجتماعية والعمل

الإدارة المركزية لتكنولوجيا المعلومات وحكم القرار



Patient Name: Dallal Aakabi Amoon Abd Al Fattah	Date : 24/04/2024
Exam : MAMMOGRAPHY BOTH	Age : 43-Year(s)
Medical ID: 4802/2021	Pat ID: 23792

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hilum and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BI-RADS 2)

Signature Dr. Nahal Hamdy

24/04/2024 4:04 PM

Patient name: Dalal Oqab Ameen

Date: 18-4-2022

Combined bilateral breast mammography and complementary ultrasound

Indication:

- ❖ Reported patient with left breast cancer on MACT
- ❖ As compared to previous study dated 27-10-2021, the current study shows,

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast Tomosynthesis)
- Bilateral breast ultrasound

Report:

- The breasts are scattered fibroglandular parenchyma (ACR B)
- Interval regression regarding the size and density of previously noted left breast findings
 - ❖ Circumscribed dense mass lesions, seen at:
 - ❖ LIC (5-7:00) Index mass lesion with related malignant microcalcifications, & currently measuring about 3.4x1.9 cm as compared to 5x3 cm, corresponding by US to deeply seated Irregular hypoechoic mass lesion.
 - ❖ Lower central (6:00) satellite, measuring 0.6x0.4 cm as compared to 1.2x1.4 cm in previous study.
 - ❖ UOO (2:00) pathological IMNL, currently is measuring 1.6x1.4 cm as compared to 1.6x1.4 cm
 - ❖ Indistinction of the previously noted other 2:00 axillary tail lesion
- Regression regarding the left pathologically enlarged axillary lymph nodes, with morphological improvement, some still showing diffuse cortical thickening 6 mm
- No right suspicious microcalcifications or speculated masses
- Complementary ultrasound show no right solid or cystic lesions
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Right Nonspecific axillary LN.

Opinion:

- Follow up status post MACT for left breast cancer showing partial radiological response as described. (BIRADS6)
- Normal right breast study (BIRADS1)

Fourteen

Dr. [Signature]

Dr. [Signature]

Path N
Sex
Age
Referr

Patient No.
Path No.
Sex
Age
Referring Doctor

28008162103469
2578/21
Female
41 Y

Request No. : 133365
Request Date : 30/10/2021
Verification Date : 10/11/2021
Financial Category : Out Patient
Department :
IHC No. :

IMMUNOHISTOCHEMISTRY REPORT

Clinical
Left breast

Clinical Data:
Left breast mass

Nature
Tru-Cut

Nature of the specimen:
Tru-Cut biopsy

Gross
5 grey

Gross Examination:
5 greyish white tissue cores 1.5cm each, Totally submitted.

Micros

• Histo
• Grad
• Angi
• Duct
• Lym
• Rese
• Add

Microscopic examination:
• Histological type: Partially autolysed specimen, invasive duct carcinoma
• Grade II
• Angiolysis: No
• Ductal carcinoma in situ: No
• Lymph node involvement: N/A
• Resection Margins: N/A
• Additional pathologic findings: N/A

Diagn

Left breast

Invasive

IHC:

ER: S

PR: S

Her-2

Ki67:

SISH

Diagnosis:
Left breast mass: Tru-Cut biopsy;
Invasive duct carcinoma grade II
IHC:
ER: Strong nuclear reaction in more than 70% of tumour cells, Positive, score (3/3)
PR: Strong nuclear reaction in about 60% of tumour cells, Positive, score (7/8)
Her-2: Strong incomplete membrane reaction in 10% of tumour cells, Equivocal, score 2+
Ki67: 15%
Pending for SISH

Prof.
Dr. A

Prof. Dr. Gama! Ahd-EI At
Dr. Abdulrahman Ibrahim

Printing Date: 10/11/2021 13:24:58

Page 1 of 2

Printed by: Amara Mohamed Younis

مركز تحقيق المخصصة
بمطابق تحقيق المخصصة

دلال
عقلى امين عبد الفتاح
منيل شبيحه
مركز ابو النورس - الجيزة

٢٨٠٠٨١٦٢١٠٣٤٨٩

١٩٨٠/١٠/١٩٨٠

J18045633



٢٠٢٢/٠٥

٢٨٠٠٨١٦٢١٠٣٤٨٩

ربة منزل

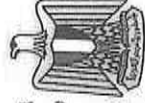
متزوجة

مسلمة

أنثى

عبدالمجيد شعبان علي طلبة سيني

البطاقة سارية حتى ٢٠٢٩/٠٥/٠٩



Patient's Name: Dlala Aqba Amin Abdul Fattah
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 1): Normal study
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Patient's Name: dalal akaby ameen
Date: 2026-04-01
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND
PELVIS REVEALED:

Known case of CA breast underwent left mastectomy. no available previous DICOM images for comparison.

I- CHEST:

- Left upper lobe subpleural fine reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Few fine bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatis lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy

مركز تحقيق المخصصة
بمطابق تحقيق المخصصة

دلال
عقلى امين عبد الفتاح
منيل شبيحه
مركز ابو النعوس - الجيزة

٢٨٠٠٨١٦٢١٠٣٤٨٩

١٩٨٠/١٠/١٩٨٠

J18045633



٢٠٢٢/٠٥

٢٨٠٠٨١٦٢١٠٣٤٨٩

ربة منزل

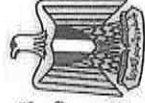
متروجة

مسلمة

أنثى

عبدالمجيد شعبان علي طلبة سيني

البطاقة سارية حتى ٢٠٢٩/٠٥/٠٩



Patient's Name: Dlala Aqba Amin Abdul Fattah
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 1): Normal study
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Patient's Name: dalal akaby ameen
Date: 2026-04-01
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of CA breast underwent left mastectomy. no available previous DICOM images for comparison.

I- CHEST:

- Left upper lobe subpleural fine reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Few fine bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatis lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy

مركز تحقيق الشخصية
بمطابق تحقيق الشخصية

دلال
عقلى امين عبد الفتاح
منيل شبيحه
مركز ابو النعوس - الجيزة

٢٨٠٠٨١٦٢١٠٣٤٨٩

١٩٨٠/١٠/١٩٨٠

J18045633



٢٠٢٢/٠٥

٢٨٠٠٨١٦٢١٠٣٤٨٩

ربة منزل

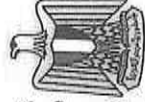
متروجة

مسلمة

أنثى

عبدالمجيد شعبان علي طلبة سيني

البطاقة سارية حتى ٢٠٢٩/٠٥/٠٩



Patient's Name: Dlala Aqba Amin Abdul Fattah
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 1): Normal study
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Patient's Name: dalal akaby ameen
Date: 2026-04-01
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND
PELVIS REVEALED:

Known case of CA breast underwent left mastectomy. no available previous DICOM images for comparison.

I- CHEST:

- Left upper lobe subpleural fine reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Few fine bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatis lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy

مركز تحقيق الشخصية
بمطابق تحقيق الشخصية

دلال
عقلى امين عبد الفتاح
منيل شبيحه
مركز ابو النعوس - الجيزة

٢٨٠٠٨١٦٢١٠٣٤٨٩

١٩٨٠/١٠/١٩٨٠

J18045633



٢٠٢٢/٠٥

٢٨٠٠٨١٦٢١٠٣٤٨٩

ربة منزل

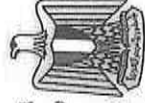
متزوجة

مسلمة

أنثى

عبدالمجيد شعبان علي ظليه سيدني

البطاقة سارية حتى ٢٠٢٩/٠٥/٠٩



Patient's Name: Dlala Aqba Amin Abdul Fattah
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 1): Normal study
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Patient's Name: dalal akaby ameen
Date: 2026-04-01
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND
PELVIS REVEALED:

Known case of CA breast underwent left mastectomy. no available previous DICOM images for comparison.

I- CHEST:

- Left upper lobe subpleural fine reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Few fine bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatis lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy

مركز تحقيق المخصصة
بمطابق تحقيق المخصصة

دلال
عقلى امين عبد الفتاح
منيل شبيحه
مركز ابو النورس - الجيزة

٢٨٠٠٨١٦٢١٠٣٤٨٩

١٩٨٠/١٠/١٩٨٠

J18045633



٢٠٢٢/٠٥

٢٨٠٠٨١٦٢١٠٣٤٨٩

ربة منزل

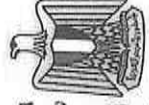
متزوجة

مسلمة

أنثى

عبدالمجيد شعبان علي طلبة سيني

البطاقة سارية حتى ٢٠٢٩/٠٥/٠٩



Patient's Name: Dala Aqba Amin Abdul Fattah
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



**RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION
SONOGRAPHY REVEALED:**

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Dense fibro-glandular densities, (ACR c).
No spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated no solid or cystic lesion.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 1): Normal study
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar

Patient's Name: dalal akaby ameen
Date: 2026-04-01
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND
PELVIS REVEALED:

Known case of CA breast underwent left mastectomy. no available previous DICOM images for comparison.

I- CHEST:

- Left upper lobe subpleural fine reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- Few fine bilateral scattered atelectatic bands.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatis lesions.

Dr.sherif Wagdy

Dr.sherif Wagdy